

***Tackling the resurgence of
preventable infectious diseases
by the loss of trust in vaccines***

World Health Organization

Table of Contents

I.	Introduction.....	2
II.	Definition of Key Terms.....	3
III.	Background Information.....	4
IV.	Major Countries and Organisations Involved.....	5
V.	Timeline of Events.....	7
VI.	Relevant UN Treaties and Events.....	8
VII.	Previous Attempts to Solve the Issues.....	9
VIII.	Possible Solutions.....	10
IX.	Sources.....	10

I. Introduction

“The challenge of public health is not only to develop effective interventions but also to ensure that societies understand and accept them.”

- William H. Foege – Epidemiologist (former CDC director)

The decline of trust in vaccination¹ has become a critical world threat to public health, contributing to the resurgence of preventable infectious diseases that were once close to elimination. Measles, whooping cough, and polio, diseases largely controlled by decades of immunisation efforts, are now resurfacing in communities where vaccination rates have fallen. While misinformation and social media amplifications fuel hesitancy in some regions, historical medical abuses and improper access to healthcare are fuelling distrust in others. To reverse this trend, a coordinated approach is needed that combines transparent communication, community engagement, and not-very-expensive provision of healthcare. Addressing the root causes of vaccine resistance, whether they come from misinformation, cultural beliefs, or systemic failures of health systems, is essential to rebuilding public trust and preventing new outbreaks of diseases that vaccines have long been proven capable of controlling.²



Emerging lessons from Africa's COVID-19 vaccine rollout

¹<https://news.un.org/en/story/2025/10/1166010>

²<https://www.unicef.org/northmacedonia/press-releases/building-trust-vaccines>

II. Definition of Key Terms

Vaccine Hesitancy

Vaccine hesitancy is defined as a motivational state of being conflicted about, or opposed to, vaccination. It exists on a continuum between absolute acceptance and total refusal. It is often fuelled by the “3 Cs” model: **Confidence** (trust in the vaccine or provider), **Complacency** (the belief that the disease is no longer a threat), and **Convenience** (the ease of access to the vaccine). This hesitation is the primary psychological reason behind the drop in immunisation rates.

Pathogen Resurgence

'Pathogen resurgence' refers to the reappearance or increase in the number of cases of a disease that was near eradication or previously under control. This occurs when the epidemiological shield provided by high vaccination coverage is breached. Common examples include recent outbreaks of measles and pertussis (whooping cough) in developed nations, where these diseases were once considered “historical” rather than active threats to public health.

Medical Disinformation

Unlike "misinformation", which is simply false information, disinformation is the intentional creation and spread of false or misleading health claims. In the context of the anti-vaccination movement, disinformation is often used as a tool to sow distrust in scientific institutions, often for political, ideological, or financial gain. This deliberate sabotage of public trust is a major hurdle for healthcare workers trying to combat preventable outbreaks.

Herd Immunity

Herd immunity is a form of indirect protection from infectious disease that occurs when a large percentage of a population becomes immune to an infection, whether through previous infection or vaccination. This reduces the likelihood of infection for individuals who lack immunity, such as newborns or the immunocompromised. When trust in vaccines falls, the vaccination rate drops below the critical threshold, leading to the resurgence of "extinguished" diseases.

III. Background Information

A. Factors Contributing to the Decline in Vaccine Confidence

1. The contemporary information environment has been transformed by the proliferation of unregulated digital communication channels, enabling the emergence of a "global infodemic". This phenomenon is characterised by the rapid spread of both accurate and inaccurate information, which frequently results in public confusion and diminishes scientific authority. Sophisticated disinformation campaigns, often amplified by social media algorithms, have exploited cultural and religious sensitivities, thereby fostering scepticism and undermining the initiatives of National Immunisation Programmes (NIPs) and international health organisations.

2. Beyond external misinformation, internal structural factors within healthcare systems have contributed to the "trust deficit". In many contexts, a lack of transparency regarding the procurement processes and the perceived influence of commercial interests in pharmaceutical development have fuelled public apprehension. Furthermore, historical instances of medical malpractice and the inequitable distribution of life-saving technologies during global health emergencies have reinforced a legacy of institutional mistrust, particularly among marginalised and indigenous populations who have historically faced systemic exclusion from quality healthcare.

B. Global Impact

1. Europe and North America

Despite possessing some of the world's most advanced healthcare infrastructures, regions in Europe and North America have experienced a marked resurgence of measles and pertussis. This trend is largely attributed to localised clusters of vaccine refusal driven by philosophical or ideological objections rather than a lack of access. In response, several Member States have introduced stringent legislative measures, such as mandatory vaccination requirements for school entry and intensified public awareness campaigns to restore herd immunity and protect immunocompromised citizens.



The President of the European Commission being vaccinated

2. Africa and Southeast Asia

In these regions, the intersection of fragile healthcare systems and targeted disinformation campaigns has created a precarious public health landscape. In certain areas, unfounded rumours regarding the safety of oral polio vaccines have resulted in temporary suspensions of immunisation drives, leading to the re-emergence of vaccine-derived poliovirus. Governments

and international partners are increasingly focusing on "community-led advocacy", utilising local religious and traditional leaders to rebuild trust and ensure that the "last mile" of vaccine delivery is met with social acceptance rather than resistance.

3. Latin America and the Caribbean

Latin America, formerly a global leader in immunisation coverage, has seen a steady decline in vaccination rates over the last decade, exacerbated by economic instability and the disruption of health services during global health emergencies. The return of measles in countries that had previously achieved elimination status highlights the fragility of regional health security. To address this, regional bodies have emphasised the integration of vaccination services into broader primary healthcare frameworks and the implementation of electronic immunisation registries to better track and mitigate gaps in coverage.

IV. Major Countries and Organisations Involved

Canada

The resurgence of vaccine-preventable diseases in Canada represents a significant regression in national health security, highlighted by the loss of its "measles-free" status in late 2025. While Canada historically maintained high immunisation rates through a robust provincial model, a growing "trust deficit" has caused several jurisdictions to fall below the 95% herd immunity threshold. This decline is largely driven by digital misinformation targeting specific socio-demographic enclaves. The federal government has responded by intensifying the "Immunisation Partnership Fund" to support community outreach and stricter surveillance. However, persistent hesitancy in provinces like Alberta and British Columbia underscores the difficulty of balancing individual autonomy with collective public health necessity.

France

France has historically navigated a complex relationship with pharmaceutical interventions, marked by deep-seated public scepticism rooted in past medical controversies. This "structural mistrust" led to a decisive 2018 legislative shift, expanding compulsory childhood vaccinations from three to eleven. While this measure initially bolstered rates, the resurgence of measles in 2025 suggests that mandates alone cannot fully neutralise organised anti-vaccination movements. The French Ministry of Health has since prioritised "institutional transparency", launching platforms to provide declassified safety data to the public. France serves as a critical case study. Demonstrating that health security requires a multifaceted strategy of both legal enforcement and proactive, transparent communication.

Romania

Romania currently faces a public health emergency, reporting some of the highest morbidity rates for vaccine-preventable diseases in Europe. The crisis is fuelled by a legacy of institutional mistrust and the heavy influence of unverified medical narratives on social media. This resurgence has strained national infrastructure, revealing vulnerabilities in reaching marginalised rural populations. In response, the government introduced a comprehensive vaccination law to improve provider accountability and stabilise vaccine supplies. However, the situation remains precarious due to significant social opposition. Consequently, international technical assistance and regional cooperation remain essential for Romania to stabilise immunisation coverage and mitigate localised information campaigns.

Pakistan

Pakistan occupies a challenging position in the global effort to eradicate preventable diseases, particularly polio. The resurgence of infections is inextricably linked to socio-political dynamics where immunisation efforts are hampered by security concerns and targeted disinformation. Militant opposition and unfounded rumours have led to significant "refusal clusters" and tragic violence against frontline health workers. In coordination with international partners, the Pakistani government has implemented a "Community Integrated Approach", deploying local health ambassadors to build trust at the household level. The success of Pakistan's health security is paramount, as the persistence of the virus within its borders remains a major transboundary threat to global health.

UNICEF (United Nations Children's Fund)

As a lead agency in global vaccine procurement and distribution, UNICEF plays a vital role in addressing the resurgence of preventable diseases by focusing on the "demand side" of immunisation. Beyond logistics, UNICEF works to rebuild trust by supporting community-based social and behaviour change programmes. By identifying "zero-dose" children in vulnerable regions, the organisation provides the resources necessary to strengthen local health systems and counter misinformation. UNICEF's mandate is to ensure that life-saving vaccines reach the most marginalised populations, fostering social mobilisation and community engagement to rebuild the "global umbrella of immunity" and protect every child from eradicated pathogens.

V. Timeline of Events

Date	Description of event
1974-1980	The WHO launched the Expanded Programme on Immunisation (EPI) in 1974. The official eradication of *Smallpox in 1980 solidified public trust in vaccines as the ultimate tool for global health security.
1998	The publication of a fraudulent study in The Lancet (later retracted) falsely linking the MMR vaccine to autism. This event is cited as the “ground zero” for the modern anti-vaccination movement and the decline in trust.
2000-2012	The launch of Gavi (The Vaccine Alliance) in 2000 expanded immunisation in low-income countries. Simultaneously, the scientific community began actively fighting “urban myths” that were beginning to lower coverage rates in Western nations.
2014-2019	Major measles outbreaks occurred in the US and Europe as herd immunity dropped below 95%. Social media algorithms accelerated the spread of misinformation, transforming vaccine hesitancy from a *niche concern into a mainstream global issue.
2019	The World Health Organisation officially listed “vaccine hesitancy” as one of the top ten threats to global health, warning that progress against preventable diseases was in danger of being reversed.
2020-2022	The rapid development of vaccines saved millions of lives but was met with unprecedented political polarisation. Distrust in authorities and vaccine mandates fuelled new resistance movements, inadvertently disrupting routine childhood immunisation schedules.
2023-present	A global effort to recover doses missed during the pandemic and combat the reappearance of *polio and *diphtheria in conflict zones. The focus shifted toward “pro-equity” strategies and empathetic, community-led communication to rebuild trust.

*Smallpox – A highly contagious, often fatal viral disease characterised by high fever and a distinct, blistering skin rash.

*Niche – Refers to a specialised or specific role, position, or segment within a larger environment.

*Polio – A highly infectious disease caused by a virus that attacks the body's nervous system.

*Diphtheria – A serious, highly contagious bacterial infection caused by *Corynebacterium diphtheriae*. It primarily attacks the mucous membranes of the nose and throat.

VI. Relevant UN Treaties and Events

★ Immunization Agenda 2030 (IA2030)

- This is the current global "North Star" for vaccines. Unlike previous plans, it specifically identifies "**Commitment and Demand**" as a core strategic priority to combat the loss of trust.

★ Political Declaration on Universal Health Coverage (2023)

- Approved by world leaders at the UN General Assembly, this declaration reaffirms that health is a human right and specifically addresses the post-pandemic trust crisis.

★ Amended International Health Regulations (IHR, 2024)

- The IHR is a legally binding treaty that defines how 196 countries must handle disease outbreaks. The 2024 amendments were specifically designed to fix gaps exposed by COVID-19.

★ The Global Compact on Refugees (GCR)

- While not a "vaccine treaty" per se, this is the primary UN framework for managing health in conflict zones.

VII. Previous Attempts to Solve the Issues

1. Restoring Public Trust through Social Listening

- To combat the misinformation, organisations like WHO and UNICEF have developed sophisticated social listening systems. These tools track trending concerns and "rumour logs" in real-time, allowing health authorities to address specific myths before they go viral. Instead of traditional top-down messaging, this strategy focuses on "pre-bunking" educating the public on how to identify manipulation tactics used by anti-vaccine campaigns.

2. Community-Led Engagement and Trusted Messengers

- Recognising that government agencies are sometimes viewed with scepticism, the UN has shifted its focus to "trusted messengers". This involves training local religious leaders, teachers, and community health workers to lead discussions on immunisation. By meeting people in familiar environments and using culturally sensitive communication, health systems can bridge the gap between scientific evidence and local concerns.

3. The Immunization Agenda 2030 (IA2030)

- This global strategy, co-created by the WHO, UNICEF, and Gavi, aims to ensure that "no one is left behind". It focuses on reaching "zero-dose" children—those who have not received a

single vaccine—by integrating immunisation into broader primary healthcare services. By making vaccines part of a holistic health package (including nutrition and clean water), providers can build long-term relationships and trust with marginalized families.

4. Strengthening Digital Literacy and Transparency

- In response to declining trust, several countries and the European Centre for Disease Prevention and Control (ECDC) have increased transparency regarding vaccine safety monitoring. Platforms like the "Vaccine Safety Net" provide easy access to peer-reviewed data. Educational initiatives now include "digital hygiene" training for parents, helping them navigate conflicting online information and verify sources through official government health portals.

VIII. Possible Solutions

★ Engaging Trusted Messengers

- Partnering with local religious leaders, teachers, and community health workers rather than just government officials to deliver vaccine education in culturally familiar environments.

★ Increasing Data Transparency

- Sharing real-time, peer-reviewed safety data through global networks like the WHO's Vaccine Safety Net to rebuild institutional credibility through radical honesty about side effects and efficacy.

★ Improving Accessible Delivery

- Investing in needle-free technologies (like patches) and better "cold-chain" storage to reach remote areas and reduce the physical barriers or fears associated with traditional injections.

Questions Delegates Should Consider During Research

- How has the return of diseases like measles or polio affected my country's health system?
- What are the specific cultural or political roots of vaccine hesitancy in my region?
- Does my country use mandates for school entry, or is vaccination strictly voluntary?
- How does my country's legislation address the spread of medical misinformation online?
- What role does my country play in global vaccine equity and reaching "zero-dose" children?

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